

ORIGINAL ARTICLE

Beyond safety net value(s): Tourist hotel rooms for people experiencing homelessness

Naomi C. Schoenfeld 

Department of Social and Behavioral Sciences,
University of California San Francisco, San
Francisco, California, USA

Correspondence

Naomi C. Schoenfeld, Department of Social and
Behavioral Sciences, University of California San
Francisco, San Francisco, California, USA.
Email: Naomi.Schoenfeld@ucsf.edu

Funding information

Social Science Resource Council (SSRC)

Abstract

This article examines the shape of care and value through an ethnographic study of an intensive, temporary housing intervention for people experiencing homelessness in San Francisco, California, during the COVID-19 pandemic. Building on a new anthropological theory of value, the results highlight the slipperiness between surveillance and care, and how value may be produced or diminished in specific, embodied ways through respite, space, privacy, and dignity. This article troubles the concept of the social “safety net,” suggesting instead that high-quality and even costly initial public investments for the poor generate public value.

KEYWORDS

COVID-19, homelessness, public value, safety net

INTRODUCTION

So, when you go out the door, you're taking the chance of dying. You're taking the chance of getting this shit. Stay your ass indoors. I know one thing. I know how to survive. I know how to do that well. I want to live. Surviving and living are two different things.

This was a distinction that Leonard, 61, returned to repeatedly as we each sat on one of the two full-size beds across from a television blaring out an old film about the Vietnam War, and I interviewed him in his COVID-19 shelter-in-place (SIP) hotel room. Dimly lit, the desk peppered with empty cans, Leonard's room had few signs of settling in. This was February 2021, and he'd been there since April 2020. Leonard is a Black man born and raised in San Francisco (SF), and he has not had a home since childhood. His comment emerged from deep experience navigating the multiple overlapping threats entailed in living unsheltered. It also reflects the explicit model of public programs for the poor: a safety net, full of holes, by design. Congregate shelters, run-down single-room-occupancy hotels (SROs), food banks,

This is an open access article under the terms of the [Creative Commons Attribution-NonCommercial-NoDerivs](https://creativecommons.org/licenses/by-nc-nd/4.0/) License, which permits use and distribution in any medium, provided the original work is properly cited, the use is non-commercial and no modifications or adaptations are made.
© 2025 The Author(s). *Medical Anthropology Quarterly* published by Wiley Periodicals LLC on behalf of American Anthropological Association.

over-stretched case management, and public hospitals are part of a public sector safety net designed around scarcity and cost-containment, intended to mitigate against the threat of “high utilization” from people who cannot pay. They provide the bare minimum for surviving, which, as Leonard notes, is distinct from the experience of living.

The pandemic wrought widespread havoc and spawned a brief sense of a shared global emergency. Yet illness, suffering, and death followed uneven and racialized distributions of risk and resources (Manderson et al., 2021). Despite seven decades of increasing government emergency preparedness in the United States (Collier & Lakoff, 2021), well-funded planning infrastructures failed to adequately respond to the pandemic emergency. Instead, responses involved significant improvisation, subject to changing political winds. While some scholars have examined the role of specific interventions, such as vaccination and masking, on metrics like viral transmission, hospitalizations, and deaths (e.g., Hartley & Perencevich, 2020), most pandemic-prompted economic and public health interventions have not been and are unlikely to ever be evaluated. For those few subject to evaluation, nearly all health interventions are subjected to extraordinarily narrow definitions of value.

In this article, I show how a pandemic intervention for people experiencing homelessness (PEH) produced value in unexpected ways. I draw an anthropological theory of embodied value together with an economic theory of public value to show how value may be found in spaces and bodies usually characterized not only as non-productive, but as draining scarce resources (Ecks, 2022; Mazzucato, 2018). In attending to the experiences of PEH living in temporary housing in tourist hotels, this article acknowledges continuities with a body of anthropological scholarship highlighting the slipperiness between surveillance and care, while building on newer theories of value as at once embodied and social.

COVID-19, HOMELESSNESS, RACE, AND RISK IN SF

Anthropologists have shown how communication about viral risk naturalizes associations of contagion along racialized, gendered, and socioeconomic lines (Briggs, 2005). The pandemic fueled technical and social interventions aimed at decreasing viral transmission, illness, and preventing health system overwhelm, generating a public narrative in which risk was defined both by health metrics and economics. The pandemic introduced a new layer to the chronic risks faced by PEH: viral morbidity, mortality, and contagion. The sociopolitical category of homelessness shifted from a usual state of chronic crisis toward a novel red alert threat-level of acute crisis. A broad set of new local, state, and national initiatives rapidly rolled out to mitigate viral risk both to and from PEH, bypassing many bureaucratic processes associated with the public sector. These programs were quickly subject to processes of (e)valuation, shaped by commitments to lean public services and a scarcity model of cost-containment.

A city of extremes and contradictions, SF is multiple: progressive mecca, epicenter of LGBTQI culture, and playground for Silicon Valley billionaires, where tech headquarters overlook sidewalks crowded with people sleeping outside. In April 2020, an outbreak in SF's largest homeless shelter panicked local leaders and advocates, spurring what became the SF SIP Hotel Program, where 7000 rooms were leased to house a population that hovers close to 9000 (Avery, 2020; Imbert et al., 2021; Robyns-Kresich, 2021). In the early months of the pandemic, SF was a model city for low viral transmission, garnering national headlines for its early decision to enact an SIP ordinance on March 16, 2020 (Allday, 2020; sfgov.org, 2020). SF's SIP hotel program was initially expected to last 6 months. Threats of closure started in December 2020, but the program ran through December 2022.

While homelessness is characterized by multiple, overlapping forms of structural violence, systemic and structural racism are central to the disproportionate impact of homelessness on Black/African American communities. Housing insecurity is a core manifestation of anti-Black racism in the United States. SF's unique brand of violence includes but is not limited to the decimation of Black communities in the Fillmore, systematic neglect of communities in the Bayview, destruction of the manufacturing sector, and racialized policing. In SF, the overall Black/African American population has fallen to

approximately 5% (census.gov, 2022), while comprising close to 40% of the population of PEH (Rhee & Rosenhack, 2021; Tides, 2024). These demographics were reflected in the SF SIP hotel project (Abbs et al., 2023). Sociologist Craig Wilse (2015) argues that narratives of public sector scarcity depict the condition of housing as a racialized, economic burden on city resources. In this context, framing management of resources for PEH as a question of cost containment serves the functions of structural racism, obscuring root causes of homelessness and inverting the framing of the economic burden of cause and effect.

THE STUDY

This article draws on a subset of data from an Institutional Review Board (IRB)-approved mixed-methods study, where detailed methodology, quantitative data and additional qualitative data are described (Abbs et al., 2023). I followed the emerging local data and corresponding public health guidelines on test positivity, case rates, and hospital utilization. In addition to my research on the SIP program, I worked as a street medicine clinician, providing medical care to PEH during the pandemic, assessing for COVID-19 symptoms and testing. This research also builds on over two decades of my clinical experience working with homeless, transiently housed, and formerly chronically homeless persons residing in SROs, some of which are permanent supportive housing (PSH). Most of my work focused on caring for the chronic conditions plaguing many who are unsheltered: harms related to substance use, mental illness, skin infections, diabetes, heart failure, and non-COVID infectious diseases like HIV and Hepatitis C. In the early months of the pandemic, I was also asked to help find people on the streets who met a shifting set of CDC criteria of vulnerability to COVID-19, including older age, underlying respiratory diseases, and immunocompromise.

The data sources used in this article include: (1) field notes; (2) digital voice recordings of interviews; (3) transcriptions and coded interviews; and (4) materials from social media, local news articles, and blogs. We obtained verbal consent for all participants. To maintain confidentiality, we created pseudonyms and obscured personal details. Two physicians who emerged as locally public figures in the media were interviewed, and they agreed to be identified, signing written consents.

I conducted research during the first year of the pandemic alongside my co-investigators on our larger, mixed-methods study. I directed and conducted all parts of the research presented in this article, with the exception of one interview conducted by a colleague with training in ethnographic interviewing. Data presented here includes participant observation during hotel placement, clinical encounters, interviews with residents and staff, and media analysis. We conducted semi-structured interviews with 27 SIP hotel residents, nine clinical staff, and four community-based organization (CBO) staff, recruiting through purposive and snowball sampling. Of the 27 residents interviewed, 12 identified as African American, eight as white, three as Asian/Pacific Islander, two as Latinx, and two chose not to identify. The hotel residents ranged in age from 18 to 72, with a median age of 55. Sixteen residents identified as male, 11 identified as female, including one who identified as transfemale. Demographic information was not collected for clinical or CBO staff. A subset of 13 residents consented to qualitative analysis of their electronic medical records. When interviewees consented to audio recording, interviews were transcribed in their original language (English or Spanish).

I used inductive, iterative, manual open coding of field notes, transcripts, and social media. Interviews began with seven questions related to residents' experiences in the SIP hotels. Open coding isolated initial themes guiding further observations, interviews, and media collection. I used axial coding for relationships between themes, focused coding for important patterns, and selective coding around core themes, refining questions based on ongoing analysis (Corbin & Strauss, 2015). A subset of interviews was independently coded by the anthropologist co-author from the larger study. Initial themes were presented to community members and stakeholders for early feedback and refinement of questions and analysis.

CARE/CONTROL, RISK/VALUE

Interventions aimed at PEH framed as care are often received as systems of surveillance and control. Ethnographies of homelessness in SF detail the profound, everyday violence experienced by PEH navigating healthcare and social services, and the policing of public space in the “compassionate city” (Bourgois & Schonberg 2009; Lopez 2020). In her ethnographic research with women experiencing homelessness and using substances, Andrea Lopez draws on Achille Mbembe to show how SF creates “necropolitical care assemblages,” where unsheltered women are forced to continually imagine their own death as they manage the risk of actual death in their engagements with care.

Homelessness in SF, as in many major cities, exists in proximity to hotel life. Paul Groth’s (1994) history of residential hotels in SF shows how the SRO has long provided an important resource for economically marginalized individuals. Groth connects the longstanding tendency to frame hotel housing as social pathology as an ongoing factor in homelessness. Kelly Knight’s (2015) ethnography focuses on sex workers in SF’s daily rent hotels. She highlights how “neurocrats” wield power to grant or withhold benefits through the disability economy, marshaling psychiatric diagnoses to render marginal forms of disability aid available in the setting of neoliberal scarcity. These observations speak to the risk for a temporary housing program to intensify layers of systemic oppression and trauma for those they are intended to serve.

In the SF landscape of homeless politics, imagined viral spread among and from PEH spurred a *viropolitical* response: management of populations according to consideration of transmissibility (Nguyen, 2009).¹ Viropolitics, in early COVID-19, functioned as a top-down management of populations fueled by pandemic emergency conditions: maneuvering quickly and often haphazardly, responding to rapidly changing assessments of economic and health indicators. At the same time, a related pragmatic logic emerged, spurring PEH and their advocates to capitalize on the pandemic conditions and demand better, more humane resources and conditions. The SIP intervention enshrined a limited citizenship on the usually marginalized and excluded: a right not to die from COVID-19; while protection from other forms of death, such as overdose, exposure, or violence, was not similarly prioritized.

The rapid creation of the SIP hotel program bypassed usual bureaucratic machinations, fueled by fear of contagion and the threat of health system collapse. The first year of the pandemic saw massive experiments in government response to the virus, featuring a range of intensive interventions characterized by social control and surveillance. Viropolitics in early COVID-19 crossed the political spectrum, covering a range of concerns and constituents, including varying degrees of emphasis on morbidity and mortality, health system collapse, economic devastation, and the balance of individual and collective rights and responsibilities. For historically marginalized and excluded populations, including PEH, the directive to “shelter in place” rendered one form of illness and death, from COVID-19, of urgent and significant calculable importance, while “usual” forms of morbidity and mortality associated with being unsheltered were rendered less valuable (Ayala-Colqui, 2020).

The SF SIP hotel initiative took shape in the context of predictions of grave risk of COVID-19 complications for PEH (Culhane et al., 2020), accompanied by dramatic federal funding. Similar programs emerged in other parts of California, in Oregon, Washington, Colorado, Illinois, and New York. The lives and deaths of PEH took on new meanings in the public discourse, and old fears of contagion took new shape: “flattening the curve” to avoid overwhelming the health system. The mandate to shelter in place, the early, large outbreak in the congregate shelter, the fears of broader contagion: these factors coalesced to fuel the humanitarian yet pragmatic urgency to secure temporary housing for PEH in SF. Pandemic viropolitics provided a mechanism streamlining the usually arduous bureaucratic process. Advocates knew the virus had the potential to bring serious harm to an already vulnerable population, and the pandemic offered an opportunity to permanently expand housing. After all, taking away programs is harder than never starting them. Policymakers would be called upon to make decisions about the duration and replication of the SIP program. Data would be needed to show whether the program had value.

RESOURCE UTILIZATION OR VALUE GENERATION

Policy questions regarding the success of the SIP hotel program relate to ways in which services for PEH are framed through neoliberal cost containment, where public spending is categorized as expenditure rather than productive investment (Mazzucato, 2018). Such framing insists that programs for the poor may be worthwhile investments if expenditures in one area result in savings in another, such as Emergency Department visits (Abbs et al., 2023; Fleming et al., 2022). Lara-Millan (2021) considers the paradox of decreasing fiscal resources in the setting of expanding legal demands for public health access and services, where “redistributing of the poor” becomes statecraft “in which poor people are legally and technically provided with a service, but their needs are also rearticulated into a different kind of need that produces revenue for the state or more conservative budgeting (18).” Wilse (2015) argues that medicalized models of homelessness have been packaged by the social sciences as a problem of inefficient resource allocation or nonscientific management.

Governance of homelessness is categorically defined as both non-productive and as a source of public resource utilization to be managed (Hennigan, 2019; Mazzucato, 2018). In SF, homelessness is a political flash point and fulcrum around which politicians distinguish themselves in large and small ways. Interventions for PEH are expected to be “fiscally responsible” in ways not seen for other populations (Brown, 2014; Gubits et al., 2015). This austerity demand can be read as an implicit moralism, a response to stereotypes, biases, and ignorance around causes of homelessness foregrounding individual factors such as substance use and mental health while obscuring root structural causes. Programs known as “Housing First” are frequently tasked with decreasing medical spending as a result of providing housing. PEH are framed as a population using scarce resources and not producing value.

Determining whether or not the SIP hotel program “worked” required making decisions, measurements, and comparisons: doing valuation. Ecks (2022) draws from his longstanding research on psychopharmaceuticals in India together with an analysis of pandemic-era politics to generate what he calls “embodied value theory.” He argues that “what is valuable emerges from a living body in an environment... Living means valuing, and what makes people’s lives better is valuable (28).” Antidepressants, for Ecks, are valued based on whether they improve conditions of living. Pandemic interventions could be compared, or in Ecks’s terms, “biocommensurated,” according to their impact on living organisms. Ecks argues for a universal theory of value grounded in enhancements for a living organism in its habitat.

Publicly funded programs like the SIP hotels are regarded as costs that the public must bear through taxation or other governmental funding streams.² Economist Marianna Mazzucato (2018) challenges the normative characterization of government spending as pure expenditure. Highlighting a long history of public investments undergirding later private developments, including the internet, cell phones, and pharmaceuticals, Mazzucato argues that value generated from initial government investment is “public value,” but this value is unrecognized in favor of strict market-based value calculations. Valuation, following Ecks and Mazzucato, exceeds strict market logics of fiscal responsibility and resource utilization. Instead, value may emerge from the body, and embodied value may also represent public value.

RESULTS

Federal Emergency Management Agency (FEMA), the Coronavirus Aid Relief, and Economic Security (CARES) Act, and the San Francisco General Fund provided the majority of the funding for the SIP hotel program (Abbs et al., 2023). PEH came to SIP through four routes: (1) transfer from congregate shelter after the April 2020 outbreak; (2) hospital discharge; and (3) street outreach using CDC criteria with additional vulnerability criteria developed by the San Francisco Department of Public Health (SFPDH), factoring in race, ethnicity, and gender identity. Commissioned hotels provided a room, private bathroom, three daily meals, onsite clinical care, and social services. At its peak in July 2021, SF provided temporary housing to 3877 in private hotels, motels, and trailers, with nearly 1700 additional PEH living in congregate shelters and safe sleeping sites (sfgov.org, 2021). As soon as the

program launched, questions and contestations of its efficacy emerged with varying degrees of urgency, according to deadlines around continued availability of federal funding and local political promises to create permanent housing for PEH. Unsurprisingly, these debates anchored assessments of risk around questions of cost.

The SIP initiative provided more comprehensive services and staffing than other temporary congregate shelters or existing PSH. SIP hotels were managed by CBOs with varying degrees of experience providing services for PEH. The SIP hotel medical support model followed the general design of the SFDPH congregate shelter model, where a nurse and a health worker offer support several hours per week. Prescribing clinicians (MD/NP/PA) spent at least a half day per week for the first 1.5 years of the program, with behavioral health clinicians, occupational and physical therapists available as needed.

Overdose death and value(s) in health relation calibration

A tragic and predictable outcome of the push to bring PEH who use opioids into hotel rooms by themselves to isolate and prevent death from COVID-19 was a spike in accidental overdose deaths as people were forced to use by themselves behind closed doors (Appa et al., 2021). These early deaths spurred the addition of harm reduction services including “wellness checks,” knocks ranging from every hour 24 hours per day to once per 8 hours; distribution of naloxone to reverse overdoses; supplies for safe drug consumption; and delivery of buprenorphine for opioid use disorder. The structure of isolation required by the SIP hotels created overdose danger zones for people using drugs who previously benefited from the presence of other users in the community. Wellness checks emerged as an early point of trauma for staff, most of whom intervened in overdoses, and some of whom encountered deceased residents. For some residents, wellness checks felt invasive. For others, checks were a welcome safety feature. One resident explained, “You know I use drugs, right? Sometimes crack-cocaine. Sometimes fentanyl. I’m trying to get off of it real bad, because I’ve been doing pretty good since I’ve been here... When I was on the street, there was no telling, and it was so bad. I think wellness checks is good because you have got to make sure you’re alive.”

PEH experience discrimination, neglect, and trauma from the healthcare and social services sector. Deciding to seek or accept care can be a significant challenge for people who have faced judgment for lack of clean clothes or access to regular bathing, for challenges attending appointments or following up on recommendations, and many other reasons. PEH are frequently denied access to high-quality, evidence-based care based on type or lack of insurance and clinician bias. Carefully calibrated wellness checks enacted relations of care and attention between staff and SIP residents, building trust and helping keep residents alive. Routinized wellness checks, on the other hand, could be seen as intrusive, adding another layer among many of heavy surveillance of poor and unsheltered, particularly among those who use drugs.

The first SIP residents moved directly from congregate shelter, and many of them were previously known to the nursing staff who simply moved job sites from congregate shelter to hotel/motel.

Lucy, a registered nurse with years of experience working in shelter health, observed,

we have a lot of people that it took a while of engaging with them, building trust, and then we have people who right off the bat were like, “This is what I need,” and now we’ve been working with them for a year.... We definitely have a very small handful of people that don’t want to engage with us mostly for psych reasons. ...that’s part of the challenge of knocking on people’s doors is when they’re not ready in that moment (April 2021).

Nurses in SIP hotels brought a range of experience and prior relationships with the residents. For PEH with significant histories of medical trauma, medical/nursing contact could be fraught. Mariana, a nurse hired at the onset of the pandemic, described the delicacy of balancing engagement with the privacy and personal space of PEH inside their SIP rooms.

It has taken months for me to grow these relationships. We try not to force them into care, we try to support them...whether that's harm reduction or setting up an appointment or going to pick up their medications. We say, "What are the things that you need from us?" Instead of forcing them into like, "You need to get your blood pressure under control." We try to meet them where they're at and say, "How can we support you (February 2021)?"

SIP nurses and residents made calculations about the relative value of growing these connections. This informal, relationship-based approach is challenging in settings of significant trauma and deprivation. One clinician, Pablo, described the chaos and uncertainty of working across multiple SIP sites and the risks of favoritism or uneven care, one of the challenges of scaling up the enhanced medical and social services of SIP in the emergency of the pandemic.

It was hard to even figure out what to do with my day. I was going from crisis to crisis. I worried that people were getting neglected. There are sites in which the nurses see the four people they like in a day and the other 90 people never get any care at all... there was no guidance going into this (February 2021).

Health relations between residents and nurses in SIP hotels formed from careful engagement in the face of well-placed mistrust, at times facilitating increased engagement in health-related activities like using harm-reduction supplies, blood pressure checks, and attending appointments. Health relationships generated incremental value here over time through small acts of care and consistency. Rapidly changing pandemic conditions and political priorities challenged SIP nurses to balance creativity with pragmatic solutions in the face of shifting job description and sites. Nurses showing up at the appointed time, remembering details, following up: these investments build value. Disregarding expressed values and preferences, violating privacy or considerations of personal space and dignity: these diminish value.

Respite and privacy as value

Homelessness takes a profound toll on the body: hypervigilance, exposure to the elements, violence, and more have been shown to accelerate aging, sometimes by decades (Brown et al., 2016). Leonard was born and raised in SF's Bayview district: a predominantly Black neighborhood long subject to multiple and overlapping forms of structural racism, with ensuing concentration of poverty, violence, and environmental contamination. It is also a site of resistance and resilience (Hagan et al., 2020). Leonard told me he had been homeless since he ran away at age 12. Now 61, he'd been among the first group of people transferred from a congregate homeless shelter to a private hotel room early in the pandemic. For Leonard, the SIP hotel got him closer to living, not just surviving. He explained:

Surviving is when you're sitting and you're scrapping and everything. You're out there. No babes, nothing, just try to eat through this, that, and the other. Living's making yourself healthy, giving somebody help in life, not trying to take...That's surviving. Living takes work. A whole lot. When you get better and better at it, it's like a muscle. It gets bigger and bigger, it gets stronger and stronger.

Leonard survived multiple childhood traumas, including witnessing the accidental drowning of his younger brother at age eight, an event that he experienced as transforming his mother from a loving parent into a distant and abusive figure from whom he later decided to escape. Caught up in the juvenile justice system as a teenager, he cycled in and out of jail and the streets for over 40 years. At the time of our interview, he had been living in the hotel for seven months, his longest continuous period inside in a decade. Leonard's distinction between living and surviving is value calibrated in the body, through food, health, shelter, safety, and sex. He describes living as labor, tangible, like muscle.

I've been to prison, been through the whole nine yards of ups and downs of what's been put in front of me and everything. I'm at a stage where I want to live... All the drugs and everything I used to do I don't do no more because I'm away from it. I'm healthy. Sometimes, it's something I want, it ain't good for you, but I want it. Now, I have to fight... where I was, it (drugs) was right there. You walk out the door and there it is. Here, you have to travel, but I don't like to travel too much. Out of a different environment to another different environment, you kind of perform to what that environment is... **You're a product of your environment, some of us make it and some of us don't.**

SIP hotels offered a temporal intervention in the cadence of surviving for PEH: respite from the chaos and danger of living outside or in congregate shelters. Seemingly small things like a personal bed, television, delivered meals, and a private bathroom afforded opportunities to not only rest, but to think and even plan. Respite in space and time shows up in the body, in what Ecks has theorized as embodied value (2022).

SIP hotel resident Clarence shared:

[the SIP hotel] afforded me the opportunity to get all of my medical appointments. I have a lot of stuff going on medically... I had the chance to get things straight... and organize, keep appointments... It's hard to keep appointments, be on time, and do things when you're homeless... I might not have accomplished some of the things I have (April 2021).

Identifying respite, the pause to think, to plan, as a potentially productive zone of value, is predicated on recognizing PEH as valuable citizens. Such an approach stands in stark contrast to usual evaluations of the safety net through utilization management and cost containment.

Several interviews with SIP residents highlighted the importance of a private bathroom to their sense of well-being. Veronica explains, "you have your own space, your own privacy, you don't have to share the restrooms, you don't have to hear as much nonsense and craziness (March 2021)." Lester offered a perspective drawing on his history of being homeless or unstably housed for most of his adult life. Having both lived and worked in SRO hotels with shared bathrooms, he reported that he would rather die than live in one again. His SIP hotel experience was different. He explained, "when I got here, I took a shower for four hours. I had to cleanse myself... I must have slept for two days (February 2021)."

For several SIP residents, private bathrooms became such a priority that it impacted their willingness to accept permanent housing without them. Linda explained, "I love my bathroom, and that's what I told them upstairs. I said, 'Now, when we go out and start looking for places, I could share a kitchen, but I have to have my own bathroom (January 2021).'" In subsequent efforts of city and NGO workers to provide permanent housing for SIP hotel residents, many refused permanent housing placement, as many of the older SROs on offer required shared bathrooms. In many ways, the private bathroom emerged as the *key site* of impactful components of SIP hotels in terms of its value-generating capacity for dignity and respite.

Always already homeless: Hotels on the carceral continuum of care³

Hotels have long functioned as a stopgap transitional housing for both PEH and formerly incarcerated people. Historically, this type of intervention involved minimal assistance toward stabilization in the form of cheap, run-down, and often dangerous hotels. SIP hotels differed in the mobilization of hotels, which, at the onset of the pandemic, were in the range of excellent to relatively good condition, having been recently used for tourists in SF. Over the course of the pandemic, all SIP hotel sites deteriorated significantly, from both intensive use and lack of adequate maintenance, in part due to the unanticipated and contingent plans for the project which was continually extended over a period of nearly 2 years. Most SIP hotels set up elaborate check-in and out procedures with intensive security in the lobby. The

procedures included signing in and out, having belongings examined, and body temperature checks. These extensive procedures differed at different sites and had the potential to render the SIP hotel experience more institutional, diminishing, to some degree, the value of respite and privacy. Almost all SIP hotel residents drew comparisons to carceral systems.

It's patterned like almost jail, the reporting in and out... they were like, 'you have to walk down the hallway,' give them your room number and turn in your own key. Then on your way outside, you have to stop at the lobby desk and give them your number and tell them you're leaving... They put rules above respect." (Beatrice, March 2021).

The SIP hotels were staffed by nonprofits with experience either running shelters or working in public spaces with PEH. Yet while the organizations may have had experience, rapid scale-up and hiring meant most workers had little to no experience, minimal training, supervision, or support. While some SIP residents felt cared for and comfortable with the CBO staff at the SIPs, negative experiences were discussed more often and in greater detail. "What I found is they've made a huge distinction of there's them and there's us. There's them, the staff who probably not very long ago were in the same shoes as the residents, us, the homeless that needed a place to stay...it's a better-than attitude (Janelle, April 2021)."

The structural racism leading to overrepresentation of Black communities among PEH similarly created the conditions many experienced in carceral systems. The relationship between hotels, homelessness, and carceral systems is complex and nuanced. Daily rent hotels and SROs have long been part of a constellation of unstable and often exploitative temporary shelter options for PEH (Knight, 2015). While this study did not set out to look for any relationship between the SIP hotels and the carceral system (jails, prisons, parole, probation), eight of the 27 hotel residents interviewed spontaneously drew comparisons between their experiences in carceral systems and their experience in the SIP hotels.

I spent years of homelessness, being on state parole... by definition, because those transitional hotels and things, that's not permanent housing. I was always homeless. You come out of prison, you're on parole. You wind up in one of those transitional hotels. You might be good for one week, but suffer the money... and you're back out on the street hustling... I've experienced homelessness for all my life because I've been on parole for quite a long time.

...I got some PTSD from my experience in prison, [the hotel room is] no bigger than a prison cell... People who have been institutionalized tend to live in these rooms like, "This is a jail cell." They fix them like a prison cell. I know what that looks like. Then, their actions, their daily routines, and things like that. It's like they're institutionalized, and it brings it back. It's just they have PTSD (Clarence, April 2021).

Dr. Joshua Bamberger, a lead physician involved in authorizing placement of PEH into the SIP hotels, described how design and staffing significantly impacted residents' experiences.

There's the architecture, the neighborhood, the financing... security, food, and then there's the onsite services... when you look at buildings that really seem to do great... having good, natural light that gets to the entire building... That the access to your unit is not through a gauntlet of some dehumanizing process so you don't have to be wanded or you don't have to climb over a barrier or show a particular ID. There's the way that people have been treated with dignity and love ... The thing is that when we do these things well, it's almost worse than when we do them badly, only because we can't deliver on the long term... that's why shelters, the whole dirty, noisy, scary shelters are purposeful so that people aren't going to feel let down if they leave (February 2021).

Both architectural design and staffing procedures had the capacity to render SIP hotel rooms closer or further from residents' carceral experiences, impacts experienced in greater or lesser degrees of embodied value. The conditional status of SIP hotels tethered to shifting pandemic conditions and political winds generated anxiety, also diminishing the program's value for the residents. "I keep having to remind myself this isn't going to last forever (Joan, February 2021)." Funding to purchase and operate buildings and political pressures led SF's Mayor London Breed to offer permanent housing to all SIP residents housed before December 2020. December 2020 marked the first of rolling announcements of SIP closures, which finally began in July 2021. Yet, the pandemic SIP hotels did not finally close until December 2022. Even for the residents who would benefit from housing, many struggled with the anxiety of not knowing the length of the SIP intervention and what would come next. Alan shared:

They closing this. It just hurt me to see that they ain't going to have no job, we ain't going to have no house. I'm worried that I don't end up on the street. Sick like I am... I don't need to be in no shelter. I need some kind of studio or some kind of room with a bathroom and refrigerator that I can do what I'm doing in here. My stuff (blood sugar) going to shoot sky-high. I'm going to be dead. They ain't got to worry about me because I'm going to be gone. I can't live on the street, it's over with. I'm not just talking about the cold on the street, I'm talking about my medical condition going to kill me (April 2021).

Anh described the anxieties provoked by multiple agencies offering conflicting information. She noted, "I think I got too much information for my own good because it makes me more nervous (February 2021)." Her response echoed many SIP residents who expressed anxiety and frustration as they waited to hear about housing.

SIP hotel rooms, especially at the start of the pandemic before the decay and disrepair, were a far cry from congregate shelters or the general first line of permanent housing option: SROs. Building trust in healthcare relationships, the respite and dignity of private rooms with bathrooms, these interventions produced embodied, life-enhancing value in PEH.

After dozens of false alarms, the SIP hotel project finally formally closed in December 2022. City officials reported that of the 3700 guests accommodated over the duration of the project, about 65% of the guests received permanent housing at the time of closure (hsh.gov, 2023). Much of the housing offered to the residents came from the approximately 70 SROs operated by the city, buildings in very poor condition, understaffed, and the majority without private bathrooms or kitchens (Palomino & Thadani, 2022). The program ultimately managed both to bring a new form of higher quality, temporary tourist hotel housing online and ultimately offered permanent housing to a majority of the SIP hotel residents.

From scarcity toward value

Value involves ideas and practices connecting individuals to societies. This study shows how higher quality interventions for PEH, notably exceeding safety net logics, can generate value along more expansive valences, such as Eck's (2022) theory of embodied value and Mazzucato's development of public value (2018). Even within progressive sectors or regions like SF, public sector spending on services for poor people is, at best, framed as spending in one domain to reduce costs in another area. The results of this study show how public sector spending on high-quality services, as opposed to safety net bare minimum, generates what Eck's might describe as embodied value, and what Mazzucato might frame as public value.

Social and medical services for PEH are governed by conflicting directives and a neoliberal model of public-sector scarcity. Congregate shelters exemplify the necropolitics of the safety net: a logic of bare minimum, perhaps even bare life (Mbembe, 2003; Agamben, 1998). The urgent expediency behind the SIP hotel project changed the landscape of post-pandemic resources for PEH in SF. Once extraordinarily

rare as a free option for PEH, post SIP, former tourist hotel rooms are now part of the landscape of choices, albeit with fewer services than pandemic era.

The SIP hotel program produced value in respite, respect, dignity, privacy, autonomy, and care for a population subject to multiple overlapping forms of oppression and stigmatization. Carceral design and procedures limited value in some hotels, and the looming threat of closure undermined the value of the endeavor. Wellness checks exemplified the tension between care and control. Knocks could be lifesaving at times, intrusive or threatening at others.

The pragmatic approach of PEH and their advocates advancing the campaign to open, expand and maintain SIP hotels repurposed pandemic viropolitics (Nguyen, 2009) and its focus on one acute viral threat to make demands on the state and to improve conditions for PEH, impacting chronic threats from the streets and congregate shelters. Assault, exposure, accidental overdose, trauma, and deterioration of chronic medical conditions are chronic threats, where survival is the most meaningful endpoint. In the provision of SIP hotel rooms, only one form of risk was considered: serious complication to COVID-19 infection which could overwhelm the health system. COVID-19 risk spurred this new temporary housing option, offering a level of dignity beyond the status quo of congregate shelters accepted under bare life safety net logic (Agamben, 1998). These tourist-quality hotel rooms, along with carefully calibrated care of nursing and support staff, produced value.

Policymakers, however, do not evaluate the SIP hotel program as a standalone entity, but as a piece of a small and shrinking pie following safety-net logics. That is, this type of housing is taken to necessarily come at the expense of permanent housing or other interventions meant to provide support or care. For decades, advocates of Housing First models have looked for ways to show the value of this intervention for PEH, often attempting to find ways it could save costs, for instance, by reducing high-cost healthcare utilization such as emergency room visits. Safety net thinking is scarcity politics where marginalized populations are concerned: if the state pays for housing, it must save money on something else. More recently, however, some researchers in Housing First studies have suggested that this may not be a viable metric and therefore the argument should shift from the economic valence of value to the ethical (e.g., Kertesz et al., 2016).

Pandemic viropolitics catalyzed a performative act: letting live through temporary “emergency” housing, eliding chronic forms of death and worsening risks of living outside and in congregate settings. In pandemic SF, the figure of death was the figure of data. It was precisely in that zone of documenting deaths through data dashboards that PEH came to “count.” PEH were not the only usually marginalized group to come to matter through a politics of contagion. Once again, the public spectacle of the data dashboards enabled transient and limited citizenship and inclusion. “Usual” death remained normative and invisible for PEH, particularly the unsheltered, who face high rates of morbidity and mortality from exposure to the elements, overdose, and violence. Data publicity in COVID-19 engendered a viropolitical response to forms of morbidity and mortality connected to the virus, alongside the persistence of the necropolitical state where usual death is routinized and invisible (Nguyen, 2009; Lopez, 2020).

The SIP hotel project could be read as a high-visibility bandage on the chronic wound of homelessness. Yet, this publicly funded endeavor also produced significant value for PEH: respite, a chance to think, plan, dream. In the SIP hotels, staying inside and out of the neighborhood he used to sleep in afforded Leonard multiple synergistic enhancements of living. Survival outside demands a complex mental and physical state of presentism, where distal futures, dreams, and plans cannot reach the horizon of attention. The increment from surviving to living is the distinction that a medically supported hotel room might afford PEH otherwise facing sleeping on the streets or in warehouse-like congregate safety net shelters. SIP hotels were created as an emergency response, escaping the long timescape and labyrinthine bureaucratic processes of housing creation. Early on, advocates and policymakers were looking for evidence to see whether this program “worked,” yet the criteria for such determination were both unclear and contested. How could Leonard’s distinction between surviving and living compare with cost-savings and efficiency analyses? Against a background of the risks of living in either the streets or congregate settings, residents and staff noted that the

COVID-19 virus was just a small addition to the multiple levels of risk and labor of survival faced daily by PEH.

CONCLUSION

Housing First advocates rightly insist housing is *the* solution to homelessness. Increasing the supply and range of housing options available is key to improving and one day ending homelessness. SIP hotels exceeded both bare life safety net norms of congregate shelters and usual entry-level permanent housing of run-down SROs, providing crucial conditions of privacy and dignity to PEH, value-generating affordances. Yet staffing procedures and architectural design can run the risk of limiting this value when care is delivered with surveillance and restriction. The difference between surviving and living, privacy and dignity to someone who has lived outside or in a shelter, these are sites of value. This study suggests that public innovations to improve the conditions of living for society's most marginalized and vulnerable generate value: they are productive endeavors for public good. The COVID-19 pandemic renders legible the degree to which state interventions to catastrophic threats to global public health will be directed by narrow definitions of value. As many scholars have pointed out, escalating existential threats to all life on earth posed by new pandemic threats, the climate catastrophe, and lingering nuclear threats have not been met with substantively new approaches to governmental responses. When Leonard says the SIP hotel helped "make yourself healthy...give someone help," he is talking about embodied value and public value. Following Ecks (2022) and Mazzucato (2018), but more importantly, listening to and amplifying voices like Leonard's, may help expand our framework of value in interventions for PEH. Looking for, and making visible, forms of value obscured by narrow market-based frameworks, shifts our attention from blame and stigmatization of poverty, from settling for bare-life safety nets. Expanding value in this way can contribute to a generative future of equity, perhaps even abundance.

ACKNOWLEDGMENTS

I extend my deepest appreciation to the residents and workers in the SF SIP hotel program who contributed their insights to this project. Gratitude to Carlos Martinez and Lauren Textor, comrades, and guest editors of this special issue, to Alex Nading, outgoing MAQ editor, as well as the incoming editorial staff. Thanks also to Leslie Dubbin who encouraged me to write this separate manuscript and give space to the rich ethnographic data from the project. This research was supported in part by the Social Science Research Foundations [Just Tech Program] and approved by the University of California San Francisco Committee on Human Research #20-32524.

ORCID

Naomi C. Schoenfeld  <https://orcid.org/0000-0002-9056-3346>

ENDNOTES

¹ Vin-Kim Nguyen use of the term "viro-politics" describes the phenomenon of therapeutic globalization (2009) in the HIV/AIDS epidemic. I employ the term without the hyphen, given the widespread use of the parallel concept of biopolitics.

² In 2018 in San Francisco, voters approved Proposition C, which leverages special taxes on businesses grossing over 50 million dollars per year and directs those funds toward homeless services. These funds were made available as of 2022. Similar initiatives have been proposed for settlement money from the tobacco industry and the entities held liable for the opioid crisis.

³ Shedd (2011)

REFERENCES

- Abbs, E., N. Schoenfeld, M. Lai, S. Satterwhite, S. Zhou, and J. Bamberger. 2023. Care and Complexity in Emergency Housing: An Examination of the COVID-19 Shelter-in-Place (SIP) Hotel Program to House People Experiencing Homelessness in San Francisco. *Journal of Urban Health* 100: 303–13. <https://doi.org/10.1007/s11524-022-00705-8>
- Agamben, Giorgio. 1998. *Homo Sacer: Sovereign Power and Bare Life*. Redwood City, CA: Stanford University Press.

- Allday, Erin. 2020. "San Francisco Has Handled the Coronavirus Pandemic Well: But at What Cost?" *San Francisco Chronicle*, November 9, 2020. <https://www.sfchronicle.com/health/article/San-Francisco-has-handled-the-coronavirus-15711673.php>.
- Appa, Ayesha, Luke N. Rodda, Caroline Cawley, Barry Zevin, Phillip O. Coffin, Monica Gandhi, and Elizabeth Imbert. 2021. "Drug Overdose Deaths Before and After Shelter-in-Place Orders During the COVID-19 Pandemic in San Francisco." *JAMA Network Open* 4, no. 5: e2110452. <https://doi.org/10.1001/jamanetworkopen.2021.10452>.
- Avery, Camden. 2020. "City Secures 7,000 Hotel Rooms for Unhoused San Franciscans to Safely Shelter in Place," *Hoodline*, April 10, 2020. <https://hoodline.com/2020/04/city-secures-7-000-hotel-rooms-to-allow-unhoused-san-franciscans-to-shelter-in-place/>.
- Ayala-Colqui, Jesús. 2020. "Viropolitics and Capitalistic Governmentality: On the Management of the Early 21st Century Pandemic." *Desde El Sur Revista de Ciencias Humanas y Sociales de La Universidad Científica Del Sur* 12, no. 2: 377–95.
- Briggs, Charles. 2005. "Communicability, Racial Discourse, and Disease." *Annual Review of Anthropology* 34: 269–91. <https://doi.org/10.1146/annurev.anthro.34.081804.120618>.
- Brown, Jennifer. "Housing the Homeless Saves Taxpayers' Money." *The Borgen Project*, June 2014. <https://borgenproject.org/housing-homeless-saves-taxpayers-money/>.
- Bourgois, Phillipe, and Jeff Schonbert. 2009. *Righteous Dopefiend*. Berkeley, CA: University of California Press.
- Census.gov. "U.S. Census Bureau QuickFacts: San Francisco County, California." Accessed January 18, 2022. <https://www.census.gov/quickfacts/fact/table/sanfranciscocitycalifornia,sanfranciscocountycalifornia/RHI225219>.
- Collier, Stephen, and Andrew Lakoff. 2021. *The Government of Emergency*. Princeton, NJ: Princeton University Press.
- Corbin, Juliet, and Anselm Strauss. 2015. *Basics of Qualitative Research*. Thousand Oaks, CA: Sage Publications.
- Culhane, Dennis, Dan Tregila, Ken Steif, and Thomas Byrne. 2020. "Estimated Emergency and Observational/Quarantine Bed Need for the US Homeless Population Related to COVID-19 Exposure by County: Projected Hospitalizations, Intensive Care Units and Mortality," March 20. <https://endhomelessness.org/resource/estimated-emergency-and-observational-quarantine-bed-need-for-the-us-homeless-population-related-to-covid-19-exposure-by-county-projected-hospitalizations-intensive-care-units-and-mortality/>.
- Ecks, Stefan. 2022. *Living Worth: Value and Values in Global Pharmaceutical Markets*. Durham and London: Duke University Press.
- Fleming, M., J. Evans, D. Graham-Squire, C. Cawley, H. K. Kanzaria, M. C. Kushel, and M. Raven. 2022. "Association of Shelter-in-Place Hotels With Health Services Use Among People Experiencing Homelessness During the COVID-19 Pandemic." *JAMA Network Open* 5, no. 7: e2223891. <https://doi.org/10.1001/jamanetworkopen.2022.23891>.
- Groth, Paul. 1994. *Living Downtown: The History of Residential Hotels in the United States*. Berkeley, CA: University of California Press.
- Gubits, Daniel, Marybeth Shinn, Stephen Bell, Michelle Wood, Samuel R. Dastrup, Claudia Solari, Scott R. Brown, et al. 2015. "Family Options Study: Short-Term Impacts of Housing and Services Interventions for Homeless Families." *SSRN Electronic Journal*. <https://doi.org/10.2139/ssrn.3055272>.
- Hagan, M., A. Hall, L. Mamo, J. Ramos, and L. Dubbin. 2020. "Homeplace: Care and Resistance among Public Housing Residents Facing Mixed-Income Redevelopment." *American Journal of Orthopsychiatry* 90, no. 5: 523–34. <https://doi.org/10.1037/ort0000452>.
- Hartley, David M., and Eli N. Perencevich. 2020. "Public Health Interventions for COVID-19: Emerging Evidence and Implications for an Evolving Public Health Crisis." *JAMA* 323, no. 19: 1908–9. <https://doi.org/10.1001/jama.2020.5910>.
- Hennigan, Brian. 2019. "From Madonna to Marx: Towards a Re-theorisation of Homelessness." *Antipode* 51, no. 1: 148–68. <https://doi.org/10.1111/anti.12410>.
- HSH.Gov. 2023. "Guest Ages at Exit: Guests Eligible for SIP Housing Process." Accessed February 22, 2025.
- Imbert, Elizabeth, Patrick M. Kinley, Ashley Scarborough, Caroline Cawley, Madeline Sankaran, Sarah N. Cox, Margot Kushel, Juliet Stoltey, Stephanie Cohen, and Jonathan D. Fuchs. 2021. "Coronavirus Disease 2019 Outbreak in a San Francisco Homeless Shelter." *Clinical Infectious Diseases: An Official Publication of the Infectious Diseases Society of America* 73, no. 2: 324–27. <https://doi.org/10.1093/cid/ciaa1071>.
- Kertesz, Stefan G., Travis P. Baggett, James J. O'Connell, David S. Buck, and Margot B. Kushel. 2016. "Permanent Supportive Housing for Homeless People — Reframing the Debate." *N-perspective*. Massachusetts Medical Society. <https://doi-org.ucsf.idm.oclc.org/10.1056/NEJMp1608326>.
- Knight, Kelly Ray. 2015. *Addicted. Pregnant. Poor*. Durham, NC: Duke University Press.
- Lara-Millán, Armando. 2021. *Redistributing the Poor: Jails, Hospitals, and the Crisis of Law and Fiscal Austerity*. New York: Oxford University Press.
- Lopez, Andrea. 2020. "Necropolitics in the 'Compassionate' City: Care/Brutality in San Francisco." *Medical Anthropology* 39, no. 8, 751–64. <https://www.tandfonline.com/doi/abs/10.1080/01459740.2020.1753046>.
- Manderson, Lenore, Nancy J. Burke, and Ayo Wahlberg. 2021. *Viral Loads: Anthropologies of Urgency in the Time of COVID 19*. London: UCL Press.
- Mazzucato, Mariana. 2018. *The Value of Everything: Making and Taking in the Global Economy*. London, UK: Hachette.
- Mbembe, Achille. 2003. "Necropolitics" *Public Culture* 15, no. 1: 11–40.
- Nguyen, Vinh-Kim. 2009. "Viropolitics: How HIV Produces Therapeutic Globalization." In *HIV/AIDS: Global Frontiers in Prevention/Intervention*, edited by Cynthia Pope, Renée T. White, and Robert Marlow (pp 539–50). New York: Routledge.

- Palomino, J., and T. Thadani. 2022. "S.F. Has Spent Millions to Shelter the Homeless in Run-Down Hotels. These Are the Disastrous Results." *The San Francisco Chronicle*, April 26. <https://www.sfchronicle.com/projects/2022/san-francisco-sros/>.
- Rhee, Taeho Greg, and Robert A. Rosenheck. 2021. "Why Are Black Adults Over-Represented Among Individuals Who Have Experienced Lifetime Homelessness? Oaxaca-Blinder Decomposition Analysis of Homelessness Among US Male Adults." *Journal of Epidemiology Community Health* 75, no. 2: 161–70. <https://doi.org/10.1136/jech-2020-214305>.
- Robyns-Kresich, Natalia. 2021. "Inside the SIP Hotels Saga." *48 Hills*, March 16. <https://48hills.org/2021/03/the-sip-hotels-saga/>.
- Shedd, Carla. 2011. "Countering the Carceral Continuum." *Criminology & Public Policy* 10, no. 3: 865–71. <https://doi.org/10.1111/j.1745-9133.2011.00748.x>.
- State of California sfgov.org. 2020. "Governor Newsom Announces Emergency Allocation of \$62 Million to Local Governments to Protect People Living in Project Roomkey Hotels." California Governor, November 17. <https://www.gov.ca.gov/2020/11/16/governor-newsom-announces-emergency-allocation-of-62-million-to-local-governments-to-protect-people-living-in-project-roomkey-hotels/>.
- SEgov. 2021. "COVID-19 Alternative Shelter Program". Accessed January 18, 2022. <https://www.sf.gov/data--covid-19-alternative-shelter-program-0>.
- Tides Foundation. "Racial Equity and Homelessness." All Home. Accessed March 9, 2024. <https://www.allhomeca.org/racial-equity-and-homelessness/>.
- U.S. Census Bureau QuickFacts. San Francisco County, California. Accessed January 18, 2022. <https://www.census.gov/quickfacts/fact/table/sanfranciscocitycalifornia,sanfranciscocountycalifornia/RHI225219>.
- Wile, Craig. 2015. *The Value of Homelessness: Managing Surplus Life in the United States*. Minneapolis, MN: University of Minnesota Press.

How to cite this article: Schoenfeld, Naomi C. 2025. "Beyond safety net value(s): Tourist hotel rooms for people experiencing homelessness." *Medical Anthropology Quarterly* e70033. <https://doi.org/10.1111/maq.70033>